SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Dose-dense AC → weekly paclitaxel + weekly trastuzumab

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

HER2-positive neoadjuvant or adjuvant high-risk breast cancer · NCCP 00433 · version 6 · 14-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Before surgery: treat the cancer and support the agreed surgical plan. After surgery: reduce recurrence risk. Confirm which source indication and sequence apply.

Schedule
Four AC cycles 14 days apart with growth-factor support, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total; no concurrent AC / trastuzumab.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Required blood tests and cardiac monitoring discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Doxorubicin
ANTHRACYCLINE CHEMOTHERAPY

- ☐ Low counts, infection and mouth ulcers
- ☐ Nausea, hair loss and fatigue
- ☐ Heart damage, sometimes delayed
- ☐ Severe tissue injury if the infusion leaks
- ☐ Rare later blood cancers

Cyclophosphamide
ALKYLATING CHEMOTHERAPY

- ☐ Low counts and infection
- ☐ Nausea, vomiting and hair loss
- ☐ Bladder irritation / bleeding
- ☐ Fertility effects and menopause
- ☐ Rare later blood cancers

Paclitaxel
TAXANE CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Numbness / tingling, sometimes lasting
- ☐ Hair loss, muscle or joint pain
- ☐ Infusion reaction
- ☐ Mouth, bowel, skin and nail effects

Trastuzumab
HER2-TARGETED ANTIBODY

- ☐ Heart pumping-function reduction / heart failure
- ☐ Infusion or allergic reaction
- ☐ Diarrhoea and fatigue
- ☐ Rare serious lung effects

Discuss the treatment setting, alternatives, exact schedule, common and serious drug-specific risks, fertility and pregnancy precautions, and the monitoring and urgent-contact plan.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00433 v6.

SACTCheck v0.84.18
Developed by Dr Paul O'Brien